

Customer types and bespoke information offer

Educational scenario - Product X

Advanced NSCLC launch strategy for UK HCPs, decision makers, and non-promotional patient-facing support.

Assumption: Product X has a UK marketing authorisation. All product promotion is HCP/relevant decision-maker only, final-form certified, SmPC-aligned, and includes prescribing information plus AE reporting routes.

Customer type	What they want to know	Bespoke information package	ABPI/MHRA boundary
Specialty care Thoracic oncology prescribers	Licensed place in pathway; efficacy and safety in trial-like vs real-world patients; sequencing and biomarker fit.	Certified HCP detail aid; pivotal evidence summary with limitations; SmPC/PI; AE management algorithm; reprint pack with AE statement.	HCP/relevant DM only; MA/SmPC aligned.
Specialty care Lung cancer CNS / nurse prescribers	Monitoring burden, counselling points, adherence support, side-effect triage, and what patients may ask.	Nurse education module; prescribed-patient starter checklist; AE triage guide; administration and follow-up checklist.	Patient materials factual, non-promotional, audience-labelled.
Secondary care Oncology pharmacists / formulary pharmacists	Dosing, interactions, dose modifications, storage, SACT protocol fit, procurement, and budget impact.	Formulary dossier; DDI and dose-adjustment quick guide; budget-impact model; pharmacy implementation checklist.	Economic claims substantiated and not exaggerated.
Diagnostic pathway MDT, pathology and biomarker leads	Which patients need testing, tissue/liquid biopsy workflow, turnaround time, and MDT eligibility checks.	Biomarker pathway map; eligibility checklist; sample-handling prompt; MDT discussion one-pager.	Service support transparent; no inducement to prescribe.
NHS decision makers ICB, Trust formulary and medicines optimisation	NICE/HTA status, eligible population, pathway impact, implementation effort, and auditability.	Value and implementation dossier; local pathway mapping workshop; readiness checklist; audit framework.	Relevant decision-maker materials certified/examined.
Public-facing support Patient organisations and prescribed patients via HCPs	Disease understanding, questions for clinical teams, support after prescribing, and safety signposting.	Non-branded disease awareness; NHS/charity signposting; prescribed-patient leaflet supplied via HCP; Yellow Card signpost.	No POM advertising to public; no personal medical advice.

Prescriber conversation model + success tracking

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A needs-led, evidence-balanced field approach that measures readiness and appropriate access, not volume pressure.

Conversation pathway for prescribers

1

Frame the visit

Confirm the customer is an HCP/relevant decision-maker. State role, purpose, licensed context, and that only certified materials will be used.

2

Discover before presenting

Use who/what/where/why/when questions: Which advanced NSCLC patients are hardest to manage? Where do testing or MDT decisions delay care?

3

Match the information package

Share only the package that answers the stated need: efficacy/safety, biomarker pathway, nurse support, pharmacy readiness, or access evidence.

4

Close, record, and follow up

Agree the next useful action: MDT education, formulary discussion, nurse/pharmacy training, or Medical Information follow-up. Log materials, needs, and AE/product complaints per SOP.

How success will be tracked

Reach + insight quality

- Priority account coverage
- Need captured in CRM
- Follow-up completed
- Common barriers tagged

Capability uplift

- HCP confidence pulse
- Nurse/pharmacy readiness
- Medical questions resolved
- Training attendance quality

Implementation readiness

- Local pathway status
- Formulary/protocol progress
- Biomarker TAT barriers
- Stock and SACT readiness

Compliance + safety

- 100% certified materials
- PI and AE route included
- AE/PC escalated per SOP
- 0 upheld Code breaches

Compliance spine

Program dashboard excludes prescribing-volume pressure as a primary KPI; uptake is interpreted only as appropriate, label-consistent access when supported by local governance.

1

No pre-MA or off-label promotion; stay within SmPC and UK access context.

2

Claims are accurate, balanced, fair, objective, substantiated, and current.

3

Promotional materials include PI and AE reporting statement; final form certified.

4

Public/patient content is non-promotional and does not encourage requests for a POM.